FOLFOX-6 Modified Therapy – 14 day

INDICATIONS FOR USE:

INDICATION	ICD10	Regimen Code	HSE approved reimbursement status*
Adjuvant treatment of stage II or III colon cancer after complete resection of primary tumour	C18-20	00209a	N/A
Metastatic colorectal carcinoma	C18-20	00209b	N/A
Advanced or metastatic adenocarcinoma of the stomach or gastro-oesophageal junction	C15, C16	00209c	N/A

* This applies to post 2012 indications

TREATMENT:

The starting dose of the drugs detailed below may be adjusted downward by the prescribing clinician, using their independent medical judgement, to consider each patient's individual clinical circumstances.

Colon carcinoma:

Adjuvant treatment is administered every 14 days for 12 cycles, unless the patient experiences disease progression or unacceptable toxicity. For patients with low-risk disease (T1-3, N1), adjuvant treatment may be administered every 14 days for 6 cycles (1).

Metastatic colon carcinoma treatment is administered continuously until disease progression or unacceptable toxicity develops.

Gastric/oesophageal carcinoma:

Treatment is administered continuously for a maximum of 12 cycles, unless the patient experiences disease progression or unacceptable toxicity.

Facilities to treat anaphylaxis MUST be present when systemic anti-cancer therapy (SACT) is administered.

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Admin. Order	Day	Drug	Dose	Route	Diluent & Rate	Cycle
1	1	Oxaliplatin [a]	85mg/m^2	IV infusion	500mL glucose 5% over 2 hours	Every 14 days
2	1	Folinic Acid [b] (Calcium leucovorin)	400mg/m^2	IV infusion	250mL glucose 5% over 2 hours	Every 14 days
3	1	5-Fluorouracil [c]	400mg/m^2	IV bolus	N/A	Every 14 days
4	1	5-Fluorouracil [c]	2400mg/m^2	Continuous IV infusion	Over 46 hours in NaCl 0.9%	Every 14 days

[a] Oxaliplatin is incompatible with NaCl 0.9%. Do not piggyback or flush lines with NaCl 0.9%.
For oxaliplatin doses ≤ 104mg use 250mL glucose 5%.
Increase infusion rate time to 4 – 6 hours in case of laryngopharyngeal dysaesthesia reaction.
Oxaliplatin administration must always precede the administration of 5-Fluorouracil.
Oxaliplatin may be given at the same time as Folinic Acid *(Calcium Leucovorin)* using a Y connector.
Acute neurotoxicity is common with oxaliplatin and can be precipitated on exposure to the cold therefore in this regimen patients should NOT suck on ice chips during the bolus injection of 5-Fluorouracil.

[b] Folinic Acid *(Calcium Leucovorin)* must be administered prior to 5-Fluorouracil. It enhances the effects of 5-Fluorouracil by increasing 5-Fluorouracil binding to the target enzyme thymidylate synthetase.

[c] See dose modifications section for patients with identified partial dihydropyrimidine dehydrogenase (DPD) deficiency

Note: Administration volumes and fluids have been standardised to facilitate electronic prescribing system builds.

ELIGIBILITY:

- Indications as above
- ECOG 0-2
- Adequate haematological, renal and liver status

CAUTION:

- Previous pelvic radiotherapy
- Recent MI
- Uncontrolled angina, hypertension, cardiac arrhythmias, CHF
- In patients with baseline greater than 3 loose bowel movements (BM) per day (in patients without colostomy or ileostomy)
- Symptomatic peripheral neuropathy
- Severe renal impairment (creatinine clearance < 30mL/minute)

EXCLUSIONS:

- Hypersensitivity to oxaliplatin, 5-Fluorouracil, or any of the excipients
- Pregnancy
- Breastfeeding
- Peripheral neuropathy with functional impairment prior to first cycle
- Known complete dihydropyrimidine dehydrogenase (DPD) deficiency

PRESCRIPTIVE AUTHORITY:

The treatment plan must be initiated by a Consultant Medical Oncologist.

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TESTS:

Baseline tests:

- FBC, liver and renal profile
- ECG (if patient has compromised cardiac function)
- DPD testing prior to first treatment with 5-Fluorouracil using phenotype and/or genotype testing unless patient has been previously tested
    - In patients with moderate or severe renal impairment, blood uracil levels used for DPD phenotyping should be interpreted with caution, as impaired kidney function can lead to increased uracil blood levels. Consequently, there is an increased risk for incorrect diagnosis of DPD deficiency, which may result in under dosing of 5-Fluorouracil or other fluoropyrimidines, leading to reduced treatment efficacy. Genotype testing for DPD deficiency should be considered for patients with renal impairment

Regular tests:

- FBC, liver and renal profile prior to each cycle
- Evaluate for peripheral neuropathy every cycle
- Assessment of hands and feet for palmar-plantar erythrodysaesthesia (PPE) as clinically indicated

Disease monitoring:

Disease monitoring should be in line with the patient's treatment plan and any other test(s) as directed by the supervising Consultant.

DOSE MODIFICATIONS:

Any dose modification should be discussed with a Consultant.

- **DPD deficiency:**
    - Consider a reduced starting dose in patients with identified partial DPD deficiency
    - Initial dose reduction may impact the efficacy of treatment
    - In the absence of serious toxicity, subsequent doses may be increased with careful monitoring
- Dose reductions listed in **Table 1** should be used when calculating FOLFOX dose reductions for patients with toxicities

Table 1: Dose Reduction Levels for All Toxicity

Drug	Dose Level 0	Dose Level -1	Dose Level -2	Dose Level -3
Oxaliplatin	85 mg/m^2	65 mg/m^2	50 mg/m^2	Discontinue
Folinic Acid *(Calcium Leucovorin)*	400 mg/m^2	400 mg/m^2	400 mg/m^2	Discontinue
5-Fluorouracil bolus	400 mg/m^2	320 mg/m^2	260 mg/m^2	Discontinue
5-Fluorouracil infusion	2400 mg/m^2	1900 mg/m^2	1500 mg/m^2	Discontinue

Note: Folinic acid is delayed or omitted if bolus 5-Fluorouracil is delayed or omitted

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Haematological:

Table 2: Dose Modifications for Haematological Toxicity

Prior to a Cycle (Day 1)	Toxicity		Dose Level for Subsequent Cycles	
	Grade	**ANC (x 10^9/L)**	**Oxaliplatin**	**5-Fluorouracil**
• If ANC < 1.5 on Day 1 of cycle, hold treatment, weekly FBC, maximum of 4 weeks • ANC ≥ 1.5 within 4 weeks, proceed with treatment at the dose level noted across from the lowest ANC result of the delayed week(s). • If ANC remains <1.5 after 4 weeks discontinue treatment	1	≥ 1.5	Maintain dose level	Maintain dose level
	2	1.0-1.49	Maintain dose level	Maintain dose level
	3	0.5-0.99	↓ 1 dose level	Maintain dose level
	4	<0.5	↓ 1 dose level	Omit bolus and ↓ 1 infusion dose level
	Grade	**Platelets (x10^9/L)**	**Oxaliplatin**	**5-Fluorouracil**
• If platelets < 75 on Day 1 of cycle, hold treatment, weekly FBC, maximum of 4 weeks • Platelets ≥ 75 within 4 weeks, proceed with treatment at the dose level noted across from the lowest platelets result of the delayed week(s). • If platelets remain <75 after 4 weeks discontinue treatment	1	≥ 75	Maintain dose level	Maintain dose level
	2	50-74.9	Maintain dose level	Maintain dose level
	3	10-49.9	↓ 1 dose level	Maintain dose level
	4	<10	↓ 2 dose levels	Maintain dose level

Renal and Hepatic Impairment:

[a]Table 3: Recommended dose modifications in patients with renal or hepatic impairment

Drug	Renal impairment		Hepatic impairment	
Oxaliplatin	**CrCl (mL/minute)**	**Dose**	No dose adjustment is needed	
	≥30	No dose adjustment is needed		
	<30	Consider 50% of the original dose		
	Haemodialysis	Consider 50% of the original dose Haemodialyse within 1.5 hours after administration		
5-Fluorouracil	Renal impairment	No need for dose adjustment is expected	Mild and moderate (without renal impairment)	No need for dose adjustment is expected
	Haemodialysis	No need for dose adjustment is expected	Severe	Not recommended

[a] Dose modification recommendations in renal and hepatic impairment obtained from Giraud et al 2023

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Management of adverse events:

Table 4: Dose modification schedule based on adverse events

Adverse reactions	Discontinue	Recommended dose modification
*Peripheral neuropathy Grade 2 present at start of cycle Grade 3 • First occurrence • 2nd occurrence • Persistent Grade 4	 Discontinue oxaliplatin Discontinue oxaliplatin	Reduce oxaliplatin by 1 dose level ↓ 1 dose level ↓ 1 dose level
Laryngo-pharyngeal dysaesthesia		Increase infusion time from 2 to 6 hours
Unexplained respiratory symptoms e.g. non-productive cough, dyspnoea, crackles or radiological pulmonary infiltrates	Discontinue oxaliplatin until interstitial disease or pulmonary fibrosis excluded	

*Neuropathy may be partially or wholly reversible after discontinuation of therapy; patients with good recovery from Grade 3 (not Grade 4) neuropathy may be considered for re- challenge with oxaliplatin, with starting dose one level below that which they were receiving when neuropathy developed.

Table 5: Dose modification of Modified FOLFOX-6 for diarrhoea

Prior to a cycle (Day 1)	Toxicity		Dose Level for Subsequent Cycles	
	Grade	Diarrhoea	Oxaliplatin	5-Fluorouracil
• If diarrhoea greater than or equal to Grade 2 on Day 1 of cycle, hold treatment. Perform weekly checks, maximum 4 times. • If diarrhoea is less than Grade 2 within 4 weeks, proceed with treatment at the dose level noted across from the highest Grade experienced. • If diarrhoea remains greater than or equal to Grade 2 after 4 weeks, discontinue treatment.	1	Increase of 2-3 stools/day, or mild increase in loose watery colostomy output	Maintain dose level	Maintain dose level
	2	Increase of 4-6 stools, or nocturnal stools or mild increase in loose watery colostomy output	Maintain dose level	Maintain dose level
	3	Increase of 7-9 stools/day or incontinence, malabsorption; or severe increase in loose watery colostomy output	Maintain dose level	↓ 1 dose level of IV push and infusional 5-Fluorouracil
	4	Increase of 10 or more stools/day or grossly bloody colostomy output or loose watery colostomy output requiring parenteral support; dehydration	↓ 1 dose level	↓ 1 dose level of IV push and infusional 5-Fluorouracil

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Table 6: Dose modification of Modified FOLFOX-6 for stomatitis

- Additional clinical management may be required

Prior to a cycle (Day 1)	Toxicity		Dose Level for Subsequent Cycles	
	Grade	**Stomatitis**	**Oxaliplatin**	**5-Fluorouracil**
• If stomatitis greater than or equal to Grade 2 on Day 1 of cycle, hold treatment. Perform weekly checks, maximum 2 times • If stomatitis less than Grade 2 within 2 weeks, proceed with treatment at dose level noted across from highest Grade experienced • If stomatitis remains greater than or equal to Grade 2 after 2 weeks, discontinue treatment	1	Painless ulcers, erythema or mild soreness	Maintain dose level	Maintain dose level
	2	Painful erythema, oedema or ulcers but can eat	Maintain dose level	Maintain dose level
	3	Painful erythema, oedema, ulcers and cannot eat	Maintain dose level	↓ 1 dose level of IV push and infusional 5-Fluorouracil
	4	As above but mucosal necrosis and/or requires enteral support, dehydration	↓ 1 dose level	↓ 1 dose level of IV push and infusional 5-Fluorouracil

SUPPORTIVE CARE:

EMETOGENIC POTENTIAL:

- As outlined in NCCP Classification Document for Systemic Anti-Cancer Therapy (SACT) Induced Nausea and Vomiting Available on the NCCP website

Oxaliplatin: Moderate (**Refer to local policy)**
5-Fluorouracil: Low (**Refer to local policy)**

For information:
Within NCIS regimens, antiemetics have been standardised by the Medical Oncologists and Haemato-oncologists and information is available in the following documents:

- NCCP Supportive Care Antiemetic Medicines for **Inclusion in NCIS** (Medical Oncology) - Available on the NCCP website
- NCCP Supportive Care Antiemetic Medicines for **Inclusion in NCIS** (Haemato-oncology) - Available on the NCCP website

PREMEDICATIONS:

Not usually required unless the patient has had a previous hypersensitivity.

OTHER SUPPORTIVE CARE:

Anti-diarrhoeal treatment **(Refer to local policy)**
Management of PPE (**Refer to local policy)**

ADVERSE EFFECTS:

- Please refer to the relevant Summary of Product Characteristics (SmPC) for details.

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REGIMEN SPECIFIC COMPLICATIONS:

- Please refer to the relevant Summary of Product Characteristics (SmPC) for full details

- **Gastrointestinal toxicity:** It manifests as nausea and vomiting and warrants prophylactic and/or therapeutic anti-emetic therapy. Dehydration, paralytic ileus, intestinal obstruction, hypokalemia, metabolic acidosis and renal impairment may be caused by severe diarrhoea/emesis particularly when combining oxaliplatin with 5-Fluorouracil

Oxaliplatin

- **Platinum Hypersensitivity**: Special surveillance should be ensured for patients with a history of allergic manifestations to other products containing platinum. In case of anaphylactic manifestations, the infusion should be interrupted immediately and an appropriate symptomatic treatment started. Re-administration of oxaliplatin to such patients is contraindicated

- **Laryngopharyngeal dysaesthesia:** An acute syndrome of laryngopharyngeal dysaesthesia occurs in 1-2% of patients and is characterised by subjective sensations of dysphagia or dyspnoea/feeling of suffocation, without any objective evidence of respiratory distress (no cyanosis or hypoxia) or of laryngospasm or bronchospasm. Symptoms are often precipitated by exposure to cold. Although antihistamines and bronchodilators have been administered in such cases, the symptoms are rapidly reversible even in the absence of treatment. Prolongation of the infusion helps to reduce the incidence of this syndrome

- **Venous occlusive disease:** A rare but serious complications that has been reported in patients (0.02%) receiving oxaliplatin in combination with 5-Fluorouracil. This condition can lead to hepatomegaly, splenomegaly, portal hypertension and/or esophageal varices. Patients should be instructed to report any jaundice, ascites or hematemesis immediately

- **Haemolytic Ureamic Syndrome (HUS):** Oxaliplatin therapy should be interrupted if HUS is suspected: hematocrit is less than 25%, platelets less than 100,000 and creatinine greater than or equal to 135 micromol/L. If HUS is confirmed, oxaliplatin should be permanently discontinued

5-Fluorouracil

- **Myocardial ischaemia and angina:** Cardiotoxicity is a serious complication during treatment with fluorouracil. Patients, especially those with a prior history of cardiac disease or other risk factors, treated with 5-Fluorouracil, should be carefully monitored during therapy

- **DPD deficiency:** DPD is an enzyme encoded by the DPYD gene which is responsible for the breakdown of fluoropyrimidines. Patients with DPD deficiency are therefore at increased risk of fluoropyrimidine-related toxicity, including for example stomatitis, diarrhoea, mucosal inflammation, neutropenia and neurotoxicity. Treatment with 5-Fluorouracil, capecitabine or tegafur-containing medicinal products is contraindicated in patients with known complete DPD deficiency. Consider a reduced starting dose in patients with identified partial DPD deficiency. Initial dose reduction may impact the efficacy of treatment. In the absence of serious toxicity, subsequent doses may be increased with careful monitoring. Therapeutic drug monitoring (TDM) of 5-Fluorouracil may improve clinical outcomes in patients receiving continuous 5-Fluorouracil infusions

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DRUG INTERACTIONS:

- Current SmPCs and drug interaction databases should be consulted for more information

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Version	Date	Amendment	Approved By
1	10/1/2015		Prof Maccon Keane
2	24/2/2015	Infusor table update	Prof Maccon Keane
3	01/03/2017	Reviewed	Prof Maccon Keane
4	27/09/2017	Updated with new NCCP regimen template and updated dosing for adjuvant treatment and haematological toxicity	Prof Maccon Keane
5	31/08/2018	Updated with new indication and standardisation of treatment table.	Prof Maccon Keane
6	2/03/2019	Updated management of diarrhoea	Prof Maccon Keane
7	12/02/2020	Standardisation of treatment table. Update exclusions, drug interactions and emetogenic potential	Prof Maccon Keane
8	26/02/2020	Standardisation of treatment table.	Prof Maccon Keane
9	20/08/2020	Reviewed. Updated exclusion criteria, baseline testing, dose modifications and adverse events with respect to DPD deficiency as per DHPC from HPRA June 2020.Updated adverse events regarding palmar-plantar erythrodysaesthesia	Prof Maccon Keane
9a	21/03/2022	Updated title and formatting corrections.	NCCP
9b	13/05/2022	Correction of typographical error.	NCCP
9c	23/11/2023	Formatting changes and grammatical corrections.	NCCP
9d	24/02/2025	Updated baseline tests with additional wording.	NCCP
10	31/10/2025	Updated ICD-10 codes, exclusion criteria, cautions, regular testing, renal and hepatic impairment dose modifications section updated to Giraud, adverse events and regimen specific complication section. Addition of adverse events table for stomatitis. General formatting and NCCP standardisation updates.	Prof Maccon Keane
10a	24/11/2025	Correction of error in renal and hepatic impairment Table 3.	NCCP

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